

Respiratory

R-06 Asthma / COPD / Bronchospasm BLS

PRIORITIES:

- ABCs
- Determine degree of physiologic distress:
 - Respiratory rate >20, use of accessory muscles, cyanosis, inadequate ventilation, depressed level of consciousness
- Maintain airway, provide oxygen and ventilatory support;
- Determine which causes best fit patient signs and symptoms, initiate treatment;
- Assure an advanced life support response;

Chronic Obstructive Pulmonary Disease

Chronic symptoms of pulmonary disease, wheezing, cough, decreased breath sounds, may have barrel chest.

1. Ensure a patent airway (suction as necessary).
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respiration as needed. DO NOT withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Place patient in position of comfort if conscious. If depressed level of consciousness, position on left side;
5. Assist advanced life support personnel with patient packaging and movement to ambulance after the unit arrives.
6. Consider;
 - Assist patient with his/her medications if available;
 - Limit any physical exertion or movement the patient may be attempting;
 - Loosen tight clothing;
 - Encourage patient to cough up sputum;
 - Keep patient warm, but not overheated.

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Revised 3/1/2000

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- ABCs
- Determine degree of physiologic distress:
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- Maintain airway, provide oxygen and ventilatory support;
- Determine which causes best fit patient signs and symptoms, initiate treatment;
- Assure an advanced life support response;

Acute Asthma/Bronchospasm

Acute onset of respiratory difficulty usually with a history of prior attacks, wheezes, coughing.

1. Ensure a patent airway (suction as necessary).
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respiration as needed. DO NOT withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Place patient in position of comfort if conscious. If depressed level of consciousness, position on left side;
5. Assist advanced life support personnel with patient packaging and movement to ambulance after the unit arrives.
6. Consider;
 - Assist patient with his/her medications if available;
 - Limit any physical exertion or movement the patient may be attempting;
 - Attempt to reduce patient anxiety;
 - Monitor vital signs, respiratory status and level of consciousness.

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Priorities

Airway Management

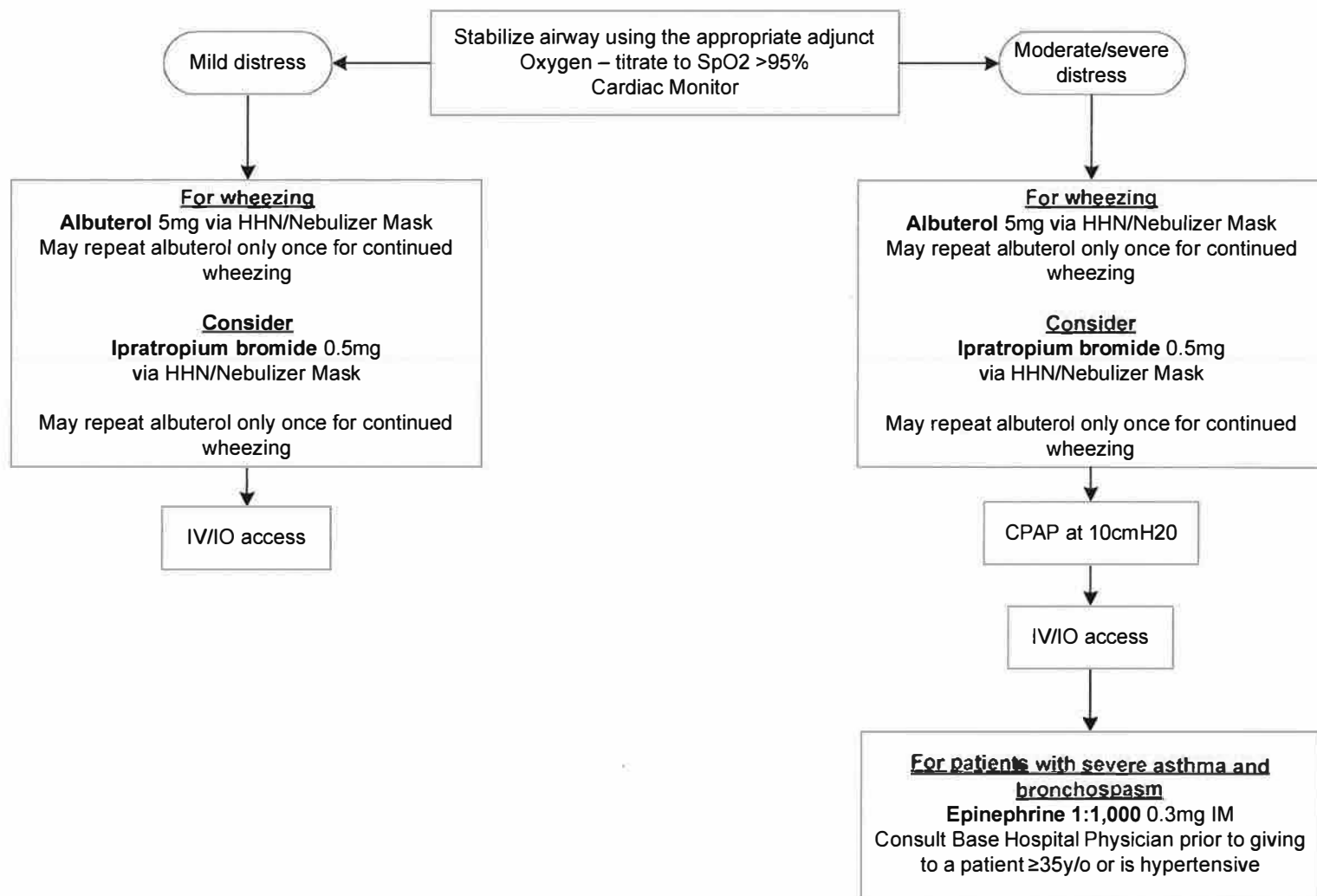
Determine degree of physiologic distress

Mild: speak full sentences, mild wheezing, fully conscious,

Moderate: speak short sentences, may have mild cyanosis, moderate wheezing, may have increased BP and HR, slight accessory muscle use, wheezing

Severe: unable to speak or speak few short sentences, ALOC, present cyanosis may be present, heavy accessory muscle use, severe wheezing

Be prepared for advanced airway use in patients with asthma



DISRUPTED COMMUNICATIONS

In the event of a "disrupted communications" situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.